

NURSING PROCESS: A TOOL FOR EVIDENCE-BASED PRACTICE

ORDER OF PRESENTATION

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- ✖ Introduction
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OBJECTIVES OF THE PRESENTATION

- ✘ By the end of this presentation, participants will be able to:
 - i. Describe the phases of the nursing process
 - ii. Utilize the nursing process in providing individualized nursing care for quality care improvement

INTRODUCTION

- ✘ Evidence-based practice is aimed at hardwiring current knowledge into common care decisions to improve care processes and patient outcomes.
- ✘ Similar to research utilization, it is “a systematic method of applying research findings to nursing practice”
- ✘ Evidence-based practice holds great promise for producing the intended health outcomes

INTRODUCTION (CONT'D)

- ✘ The current health care environment enhances discipline-specific and interdisciplinary contributions to patient outcomes
- ✘ The call for evidence-based quality improvement and healthcare transformation underscores the need for redesigning care that is effective, safe, and efficient
- ✘ A perfect fit for the nursing process

OVERVIEW OF THE NURSING PROCESS

Nursing Process

- ✖ A systematic, client-centered method for structuring the delivery of nursing care
- ✖ Systematic, rational method of planning and providing nursing care
- ✖ Characteristics include: client centered, cyclical, dynamic, interpersonal, collaborative, flexible and universally applicable.

OVERVIEW OF NURSING PROCESS CONT'D

Benefits of the Nursing Process

Nurse:

- ✖ Effective use of time and resources
- ✖ Consistent, systematic education & Professional growth
- ✖ Job satisfaction
- ✖ Defines nursing roles to outsiders
- ✖ Enhances collaboration with health team members
- ✖ Meets standards of care and the profession

OVERVIEW OF THE NURSING PROCESS CONT'D

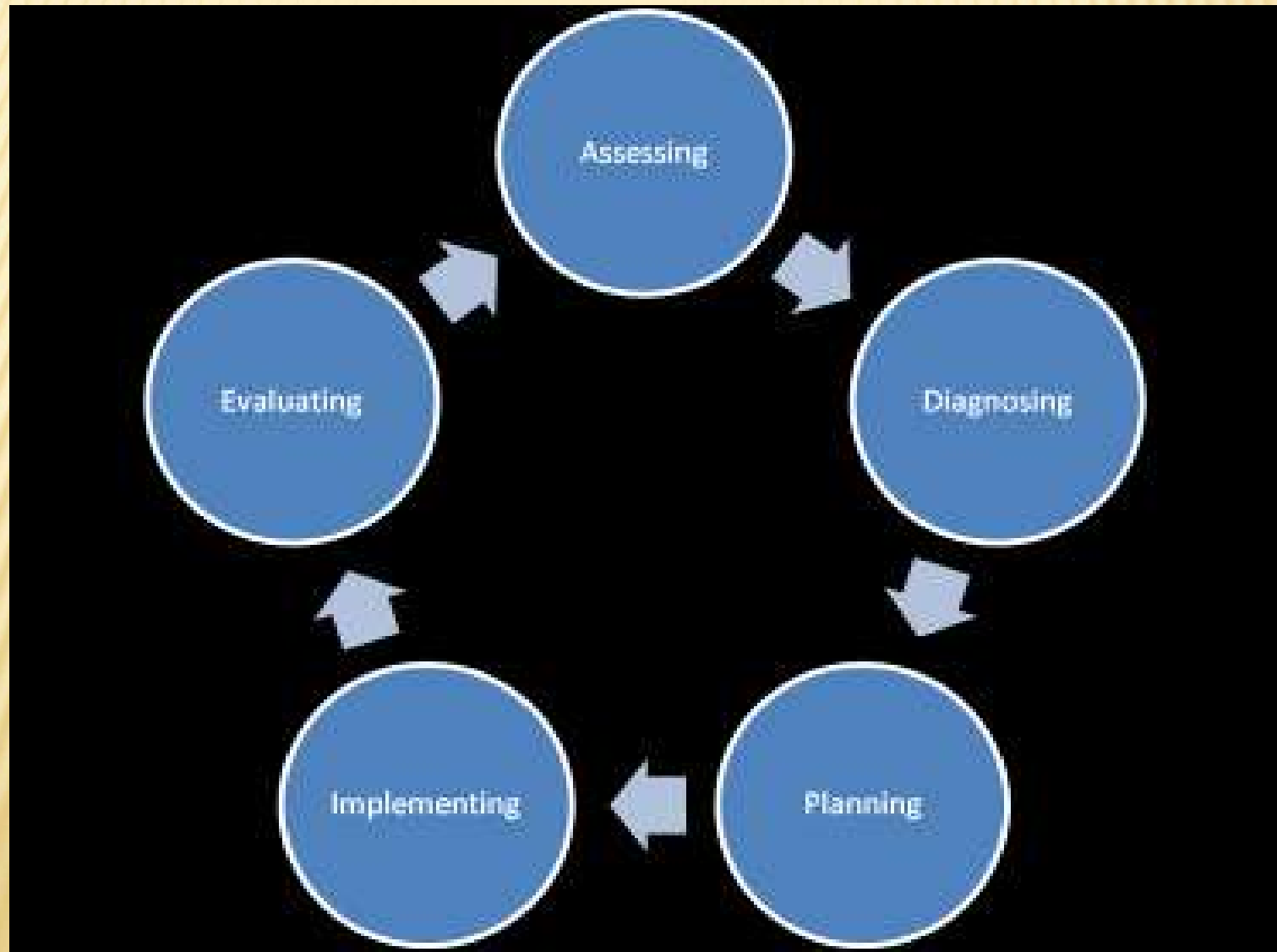
Nurse (cont'd):

- ✗ Enhances professional accountability
- ✗ Avoids legal confrontations

Patient:

- ✗ Improved quality of care
- ✗ Scientific based, holistic and individualized care
- ✗ High level of participation in care
- ✗ Continuity of care
- ✗ Responsibility for own health

COMPONENTS OF THE NURSING PROCESS



COMPONENTS OF THE NURSING PROCESS

Assessing

- ✖ Collect data
- ✖ Organize data
- ✖ Validate data
- ✖ Document data

COMPONENTS OF THE NURSING PROCESS

Diagnosing

- ✗ Analyze data
- ✗ Identify health problems, risks and strengths
- ✗ Formulate diagnostic statements
- ✗ Evaluate the quality of diagnostic statements

COMPONENTS OF THE NURSING PROCESS

Planning

- ✗ Prioritize problems/diagnoses
- ✗ Formulate objectives /outcome criteria
- ✗ Select nursing interventions
- ✗ Develop a nursing care plan

COMPONENTS OF THE NURSING PROCESS

Implementing

- ✗ Reassess the client
- ✗ Determine nurse's need for assistance
- ✗ Implement nursing interventions
- ✗ Supervise delegated care
- ✗ Document nursing activities

COMPONENTS OF THE NURSING PROCESS

Evaluating

- ✗ Collect data related to outcomes
- ✗ Compare data with outcomes
- ✗ Relate nursing actions to client objectives /outcomes
- ✗ Draw conclusions about problem status
- ✗ Continue, modify or terminate the client's care plan

ASSESSING

- ✖ First and vital step in the nursing process, done systematically and continuously to reflect changes in patient's condition.
- ✖ **Purpose:** to establish a database about the client's response to health concerns/illness and the ability to manage health care needs.

ASSESSING CONT'D

Types of Assessment:

- ✖ Initial
- ✖ Problem-focused/Ongoing
- ✖ Emergency
- ✖ Time-lapsed

Assessment methods:

- ✖ Observing
- ✖ Interviewing
- ✖ Examining

ASSESSING CONT'D

1. Collecting Data

- ✗ The process of gathering information about a client's health status
- ✗ Done systematically and continuously to reflect client's changing health status

Types of Data:

- ✗ Subjective/Covert data/Symptoms
- ✗ Objective/Overt data/Signs

ASSESSING CONT'D

Sources of data:

- ✗ Primary/Direct
- ✗ Secondary/Indirect

Characteristics of data:

- ✗ Complete
- ✗ Accurate
- ✗ Factual
- ✗ Relevant

ASSESSING CONT'D

2. Organizing Data

- ✗ Uses written/computerized format that organizes assessment data systematically
- ✗ Referred to as nursing health history/nursing assessment/nursing data base form
- ✗ Components may include available frameworks i.e. nursing and non nursing models.

ASSESSING CONT'D

Basic Components of a Nursing Health History

- ✗ Biographical information
- ✗ Reasons for seeking health care/Chief complaint
- ✗ Present illness/health concern
- ✗ Past health history
- ✗ Family history
- ✗ Environmental history
- ✗ Psychosocial and cultural history
- ✗ Review of Systems (ROS)

ASSESSING CONT'D

3. Validating data

- ✗ Double-checking/verifying data/comparing data with another source to confirm that they are accurate and factual
- ✗ Ensures assessment data is complete
- ✗ Ensures objective & related subjective data agree
- ✗ Provides additional information that may have been overlooked
- ✗ Helps differentiate between cues & inferences
- ✗ Literature review confirms that data are consistent with medical diagnosis

ASSESSING CONT'D

4. Recording Data

- ✗ Accurate documentation of all data about the client's health status is essential
- ✗ Accurate and factual documentation of all data about the client's health status
- ✗ Subjective data are recorded in client's own words and not interpreted

DIAGNOSING

- ✘ A pivotal step that requires critical thinking skills to interpret assessment data and identify client strengths and problems.
- ✘ Diagnosis is often used to describe both a process and a product.
- ✘ **Nursing diagnosis** is a statement that describes the client's actual/potential response to a health problem that the nurse is licensed and competent to treat.

DIAGNOSING CONT'D

Types of Nursing Diagnoses

- ✗ **Actual nursing diagnosis:** problem is present, and based on the presence of associated S & S e.g. Ineffective breathing pattern / Anxiety.
- ✗ **High risk nursing diagnosis:** presence of risk factors indicates that a problem is likely to develop e.g. High risk for infection

DIAGNOSING CONT'D

- ✖ **Possible nursing diagnosis:** evidence about a health problem is incomplete/unclear e.g. Possible social isolation related to unknown aetiology
- ✖ **Wellness diagnosis:** indicates a healthy response. Client desires a higher level of wellness e.g. Readiness for enhanced coping
- ✖ **Syndrome diagnosis:** associated with a cluster of other diagnoses e.g. Rape-trauma syndrome

DIAGNOSING CONT'D

NANDA nursing diagnosis /Diagnostic statement

- ✗ **Problem/Diagnostic label:** describes the client's health problem in a few words. Directs formation of goals/expected outcomes, and may suggest some nursing interventions. Need to be specific, stating area of problem e.g. Knowledge deficit (medications). May require use of qualifiers to give additional meaning e.g. Altered, Deficient, Impaired, Ineffective, Acute, Chronic etc.

DIAGNOSING CONT'D

- ✖ **Etiology (Related factors/Risk factors):** identifies one or more probable causes of the problem, gives direction to required nursing therapy and individualizes care. Identifying the right cause of the problem is important.
- ✖ **Defining Characteristics:** S & S that indicate the presence of a particular diagnostic label/problem

DIAGNOSING CONT'D

1. Analyzing data

- ✖ Comparing data with standards/norms based on knowledge and experience, to identify significant and relevant cues
- ✖ Clustering cues: involves determining the relatedness of facts and whether any patterns are present. Involves making inferences about the data.
- ✖ Nurse interprets the possible meaning of cue clusters and labels them with tentative diagnostic statements.
- ✖ Identifying gaps and inconsistencies in data: skillful assessment ensures complete data.

DIAGNOSING CONT'D

2. Identifying health problems, risks & strengths

- ✖ Determining problems and risks: the nurse and patient identify actual, risk and possible problems, and determine type of problem and the need for help in dealing with each problem.
- ✖ Determining aetiologies: the nurse examines the causal relationships between problems and their related/risk factors
- ✖ Determining strengths: They establish client's strengths, resources and abilities to cope.

DIAGNOSING CONT'D

3. Formulating diagnostic statements

- ✗ **Basic two-part statements:** Problem (P) – statement of client's response + Etiology (E)– factors contributing to /probable causes of the response. E.g.
 - Ineffective breastfeeding related to breast engorgement
 - Constipation related to prolonged laxative use
 - The two parts are joined together by related to/ associated with, merely implying a relationship

DIAGNOSING CONT'D

✗ Basic three-part statement: PES format

Problem + Etiology + Signs & Symptoms, where S&S are the defining characteristics manifested by the client (for actual problems only). The parts are joined by **related to** and **as manifested by**.

- E.g. Fluid volume deficit related to persistent diarrhoea and reduced fluid intake as manifested by dry skin and mucous membranes, reduced urine output and poor skin turgor
-

DIAGNOSING CONT'D

- ✖ **One-part statement:** Wellness and Syndrome nursing diagnoses consist of NANDA labels only. More specific so that nursing interventions can be derived from the labels.

E.gs

Rape-trauma syndrome; Effective breastfeeding

Post-trauma response; Potential for growth

Defensive coping; Health-seeking behaviours

DIAGNOSING CONT'D

Some Variations of Basic Formats

- i. Writing “unknown etiology” e.g. Noncompliance related to unknown etiology
- ii. Using “complex factors” when there are too many etiologic factors e.g. Chronic low self esteem related to complex factors
- iii. Using the word “possible” to describe the problem/etiology when more data is required e.g. Possible low self esteem related to loss of job and rejection by family

DIAGNOSING CONT'D

- iv. Using “secondary to” to divide the etiology into two making the statement more descriptive and useful
e.g. Impaired skin integrity related to decreased peripheral circulation secondary to Diabetes.

DIAGNOSING (CONT'D)

4. Evaluating the quality of diagnostic statement
 - ✖ Diagnostic Statements must be:
 - accurate
 - concise
 - descriptive and
 - specific

DIAGNOSING (CONT'D)

Guidelines for writing Diagnostic statements

- ✖ State in terms of a problem not a need e.g. Fluid volume deficit, not Fluid replacement
- ✖ State so that it is legally advisable e.g. Impaired skin integrity related to immobility, not related to improper positioning.
- ✖ Use non judgmental statements e.g. Spiritual distress related to inability to attend church services, not related to strict church rules necessitating church attendance

DIAGNOSING CONT'D

- ✗ Ensure that both parts of the statement do not say the same thing
- ✗ Ensure that the client's response/problem precedes the contributing factor or cause
- ✗ Use statements that provide guidance for planning independent interventions.
- ✗ Statements must be specific and precise e.g. Acute Pain (abdomen) related to inflamed peritoneum, not related to disease process.

DIAGNOSING (CONT'D)

- ✗ Use nursing rather than medical terms to describe both client's response/problem and probable cause.
- ✗ Do not start a diagnosis with a nursing intervention.
- ✗ Avoid using a symptom such as nausea as the problem
- ✗ Use terms generally understood by other professionals, not jargons or abbreviations
- ✗ Ensure that cause and effect are correctly stated

PLANNING

Deliberative systematic step, involves decision making and problem solving.

Types:

- ✗ Initial planning
- ✗ Ongoing planning
- ✗ Discharge planning

Steps:

1. Prioritize problems/diagnoses: high priority, medium priority, low priority. Guided by a framework like Maslow's hierarchy of needs

PLANNING (CONTD)

- ✗ **High priority** - life threatening problems e.g. loss of respiratory/cardiac functioning (Ineffective airway clearance; Fluid volume deficit; Ineffective breathing pattern)
- ✗ **Medium priority** - health threatening problems e.g. acute illness/reduced coping (Anxiety; Altered comfort)
- ✗ **low priority** – Problems that arise from normal developmental needs/require only minimal nursing support (Altered nutrition; Self care deficit; Altered sleep pattern)

PLANNING (CONT'D)

2. Formulating objectives/outcome criteria

- ✗ Set for each stated N.D.
- ✗ written in order of priority
- ✗ Derived from the problem of the N.D.
- ✗ Provide direction for interventions
- ✗ Provide sense of achievement in patient & nurse.

Objectives (goals)

- ✗ Broad statements of desired outcomes/change in patient behaviour.

PLANNING (CONTD)

- ✗ Describe what the nurse with the patient want to achieve in terms of observable patient responses rather than nursing activities.
- ✗ Begin with “Patient”
- ✗ Provide realistic time span for planned activities
- ✗ Derived from the problem clause of the nursing diagnosis e.g. High risk for fluid volume deficit
- ✗ Stated as the opposite of the problem e.g. Patient’s fluid balance will be maintained

PLANNING (CONT'D)

- ✗ Each objective is derived from only one nursing diagnosis
- ✗ Must be compatible with the work and therapies of other health professionals

Outcome Criteria

- ✗ More specific & measurable criteria used to evaluate whether the objective has been met e.g. output in balance with fluid intake, normal skin turgor, moist mucous membranes.
- ✗ Outcomes must be realistic for patient's capabilities, limitations & designated time span

PLANNING (CONT'D)

- ✗ If achieved, would be evidence that the problem has been resolved/prevented e.g. fluid volume deficit has been prevented & the objective has been achieved.
- ✗ Use observable measurable terms, avoid vague words that require interpretation/judgment e.g. patient answering questions correctly/increase in body weight
- ✗ State at least 2 outcome criteria
- ✗ Nursing diagnosis with subjective problem clause will require a subjective outcome criteria

PLANNING (CONT'D)

- ✗ Join the two parts with “as evidenced by” e.g.
- ✗ Patient’s fluid volume will be restored within 24hrs as evidenced by
 - i. Good skin turgor
 - ii. Output in balance with intake

OR

PLANNING (CONT'D)

- ✗ Restore fluid volume within 24hrs as evidenced by good skin turgor and output in balance with intake

OR

- ✗ Patient having good skin turgor and output in balance with intake within 24hrs

PLANNING (CONT'D)

3. Selecting Nursing Interventions

- ✗ Nursing activities that the nurse & patient will carry out to achieve a specific objective.
- ✗ Correct identification of the cause (2nd clause) of the problem provides framework for choosing successful nursing interventions.
- ✗ Nurse & patient establish a number of alternative interventions.
- ✗ Each strategy is reviewed based of knowledge & experience

PLANNING (CONT'D)

- ✗ Focus is on eliminating the cause in Actual or reducing risk factors in High risk N.D.
- ✗ Must include observation/assessment
- ✗ Usually 3 -5, maximum 6 are carefully chosen

Selected strategies must be:

- ✗ safe & appropriate for patient's age, health etc
- ✗ achievable with resources available
- ✗ congruent with patient's values & beliefs

PLANNING (CONT'D)

- ✗ congruent with other therapies
- ✗ based on nursing knowledge & experience
- ✗ within established standards of care

Types of Nursing Interventions

- ✗ Independent interventions
 - ✗ Dependent interventions
 - ✗ Collaborative Interventions
-
- ✗ Convert nursing interventions into orders

PLANNING (CONT'D)

Components of Nursing Orders

- ✗ Date – orders are then reviewed regularly .
- ✗ Action verb – starts the order & must be precise e.g. explain, apply, measure etc.
- ✗ Content area – the where and what of the order e.g. leg, wound, skin; bandage, lotion, sterile towel etc.
- ✗ Time element – answers when, how long/how often an action should occur e.g. daily, morning, 4hourly etc.
- ✗ Signature – accountability & legal significance

PLANNING (CONT'D)

Depending on the type of patient problem, the nurse writes the following orders:

- ✖ Observation orders e.g. record I & O hourly
- ✖ Prevention orders e.g. turn patient 2hourly
- ✖ Treatment orders e.g. dress wound daily
- ✖ Health promotion orders e.g. discuss importance of daily exercise

PLANNING CONT'D

4. Developing a nursing care plan

- ✗ Nursing Care Plan - written guide that organizes information about patient's care into a meaningful whole.
- ✗ Includes actions nurses must take to address patient's N.D. & meet stated objectives.
- ✗ Drawn as soon as admitted and constantly updated throughout hospitalization

PLANNING (CONT'D)

Purposes of NCP:

- ✖ Provides direction for individualized care
- ✖ Provides continuity of care
- ✖ Provides direction about what needs to be documented on patient's progress notes
- ✖ Serves as guide for assigning staff to provide care
- ✖ Serves as guide for reimbursement from medical insurance companies

IMPLEMENTING

- ✖ The nurse puts the nursing care plan into action.
- ✖ Consists of doing, delegating & recording.
- ✖ The nurse carries out independent, dependent & collaborative functions.
- ✖ Patient participation are encouraged depending on health status.
- ✖ Ongoing assessment occurs simultaneously with implementing

IMPLEMENTING (CONTD)

Process of Implementing

1. Reassessing the client

- ✗ The nurse finds out whether the intervention is still needed

2. Determining need for nursing assistance

- ✗ physical, knowledge/skill

3. Implementing nursing interventions

- ✗ Caring, communicating, helping, teaching, counseling, leading and managing

IMPLEMENTING (CONT'D)

4. Supervising delegated care

- ✖ Assign and delegate care to other nurses
- ✖ Supervise and evaluate nursing activities of others

Guidelines for implementing nursing strategies

- ✖ Base nursing actions on scientific knowledge, nursing research and professional standards of care.
- ✖ Understand clearly orders to be implemented.

IMPLEMENTING (CONT'D)

- ✖ Adapt nursing actions to the individual client, should be holistic and safe.
- ✖ Often actions require teaching, support and comfort.
- ✖ Nursing actions should respect the dignity of the patient and enhance self esteem.

IMPLEMENTING (CONT'D)

5. Document nursing activities

- ✗ Record interventions & patient's responses in nurses notes
- ✗ Record **after** and not before the activity is carried out.
- ✗ Routine activities may be recorded immediately after carrying them out or at the end of shift
- ✗ Communicate nursing actions verbally as well.

EVALUATING

- ✗ Planned ongoing, purposeful activity to determine
 - i. client's progress towards goal achievement, and
 - ii. effectiveness of the Nursing Care Plan.
- ✗ Effectiveness depends on preceding steps

Types of evaluation

- ✗ Ongoing
- ✗ Intermittent
- ✗ Terminal

EVALUATING (CONT'D)

Steps in evaluating

1. Collecting data related to outcomes

- ✗ Subjective & objective data, recorded concisely & accurately in order to make conclusions about objective achievement.

2. Comparing data with outcomes

- ✗ Make judgments about objective achievement & write an evaluative statement – conclusion + supporting data e.g. Objective fully met + achieved outcome criteria.

EVALUATING (CONT'D)

3. Relating nursing actions to client outcomes

- ✗ Determine whether nursing actions relate to/led to goal achievement.

4. Drawing conclusions about problem status

- ✗ Use judgment about client outcome to decide whether plan was effective in resolving, reducing or preventing client problems.

EVALUATING CONT'D

5. **Continuing, modifying or terminating the care plan:** If objective is fully met, a hi-liter is used to draw a line through or “discontinue” is written across and dated; otherwise the entire care plan is reviewed by going through the stages of the nursing process all over again.

CLINICAL SITUATION A

Justice Inkoom, 22yrs and a 2nd year university student is admitted to the male surgical ward with a diagnosis of Peritonitis 2^o to typhoid perforation. On the morning of first day post-op you met him with a temperature of 39.9°C, vomiting, sunken eyes, dry skin with poor turgor, and he complains of abdominal pain. He appears anxious.

CLINICAL SITUATION A CONT'D

- ✖ ?? Justice's health problems
- ✖ ?? Nursing diagnoses
- ✖ ?? Objectives and outcome criteria
- ✖ ?? Nursing Interventions
- ✖ ?? Evaluation of care

PATIENT'S IDENTIFIED PROBLEMS

- ✗ surgical wound
- ✗ high body temperature (39.9°C)
- ✗ abdominal pain
- ✗ dehydration
- ✗ anxious
- ✗ excessive gastric drainage (780ml/24hrs)

NURSING CARE PLAN

Date/ Time	Nursing Diagnosis	Objective / Outcome Criteria	Nursing Orders	Nursing Interventions	Date/ Time	Evaluation	Sig
14/5/ 14 10.45 am	Fluid volume deficit related to loss per NG drainage (740mls/24hrs) and fever (39.9°C)	Patient's fluid volume will be restored within 24hrs as evidenced by: . moist skin and mucous membranes. . good skin turgor.	.Assess degree of dehydration twice dly. . Give patient IV fluid 3L/24hrs as ordered. . Monitor and record fluid intake and output and report any abnormality. . Check vital signs 4hrly and report any abnormality. .Tepid sponge patient when temperature is above 38° C.	.Degree of dehydration was assessed twice dly. . IV fluid 3L/24hrs was administered as ordered. . Fluid intake and output were strictly monitored and recorded and any abnormality reported. . Vital signs were checked and recorded 4hrly .Patient was tepid sponged when temperature read above 38° C.	15/5/ 14 11am	Goal fully met. Patient's skin and mucous membranes were observed to be moist and had good skin turgor.	HA

NURSING CARE PLAN

Date/ Time	Nursing Diagnosis	Objective/ Outcome Criteria	Nursing Orders	Nursing interventions	Date/ Time	Evaluation	Sig
14/5/ 14 10.45 am	Pain (abd.) related to surgical intervention	Patient's pain will be relieved within 72hrs as evidenced by: .patient verbalizing that pain is relieved .nurse observing patient is cheerful	.Assess pain intensity using numeric rating scale. .Assist patient into the most comfortable position. .Provide diversional therapy e.g. show TV programme of choice .Serve analgesic – IM Pethidine 100mg 8hrly	.Pain intensity was assessed using numeric rating scale 0-10 12hrly .Patient was assisted into low fowler's position. .Patient was shown African movie of his choice for diversion .IM Pethidine 100mg 8hrly was served as ordered.	17/5/ 14 11am	Goal fully met. Patient verbalized relief of pain and was seen to be cheerful	HA

NURSING CARE PLAN

Date/ Time	Nursing Diagnosis	Objective/ Outcome Criteria	Nursing Orders	Nursing Interventions	Date/ Time	Evaluation	Sig
14/5/ 14 11.55 am	Anxiety related to acute nature of illness and unknown outcome of condition	Patient's anxiety will be relieved within 48hours as evidenced by: .relaxed facial expression .Patient interacting with other inmates of the ward. .Patient verbalizing he is no more anxious	.Assess patient's level of anxiety and physical reactions 6hrly. .Reassure patient of the competence of the health team. .Explain every procedure and routines to be carried out on patient. .Encourage patient to express his fears and concerns .Introduce to him a patient who had been through similar situation and surgery and is doing well.	.Patient's anxiety level and physical reactions were assessed 6hrly. .Patient was reassured of competence of the health team. .Every procedure/ routine was explained in simple language to patient. .Patient was encouraged to express his fears and concerns .A patient who had similar surgery and doing well was introduced to him	16/5/ 14 12nn	Goal fully met. Patient said he was no more anxious and was observed to be relaxed and interacting with other inmates of the ward.	HA

REFERENCES

- ✖ Breman, A., Snyder, S., Kozier, G., Erb, G. (2008). *Kozier and Erb's Fundamentals of Nursing: Concepts, Process & Practice (8th ed.)*. New Jersey: Pearson Prentice Hall.
- ✖ Doenges, M. E. & Moorhouse M. F. (2008). *Application of Nursing Process & Nursing Diagnosis: An interactive text for Diagnostic Reasoning (5th ed.)*. Philadelphia: FA Davis Company.

- ✖ Doenges, M. E., Moorhouse, M. F. & Murr, A. C. (2006). *Nurse's Pocket Guide: Diagnoses, Prioritized Interventions and Rationales* (10th ed.). Philadelphia: F. A. Davis Company
- ✖ Kozier, G., Erb, G., Breman, A., Snyder, S., Lake, R. & Harvey, S. (2008). *Fundamentals of Nursing: Concepts, Process & Practice* (8th ed.). England, Toronto, New York, Paris: Pearson Education, Nursing & Health.

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- ✖ Questions / Contributions
 - ✖ Summary
 - ✖ Conclusion

THANK YOU